

Registry fact independently re-verified (not taken on either bench's word). Options chain remains blocked — that stays a stated gap.

PITCH — GPCR — VERDICT

PROPOSED — PENDING ADJUDICATION — No entry today at \$47.34; the stock is roughly fairly priced against our own scenario math (EV \$49.6 vs tape \$47.34, +4.8%), and both the stock and the options are unattractive, so we pre-register reopen levels and arm sensors instead of buying.

What we believe

Structure Therapeutics is developing aleniglipron, a pill you swallow once a day that does what Ozempic and Zepbound do by injection. In its Phase 2 trial it produced 15.3% absolute weight loss at 44 weeks, and the extension study showed 16.2% at 56 weeks with the curve still going down. That is, on the evidence available, the most effective oral weight-loss drug in development. It also cleared the trap that killed Pfizer's version of this drug: the liver. The company reports no drug-induced liver injury, no persistent enzyme elevation and no heart-rhythm signal across more than 625 people dosed.

We also believe three things that cut against owning it today:

1. **The money arrives too late.** The trial that decides this company finishes in **September 2028**. We verified that directly against the trial registry rather than accepting it from either side of our own debate: ACCOMPLISH-1, 3,600 patients, primary endpoint at week 76, primary completion September 2028, sponsor Gasherbrum Bio (a wholly owned Structure subsidiary) — [ClinicalTrials.gov NCT07654361](https://clinicaltrials.gov/ct2/show/study/NCT07654361). A drug approval would follow in 2029 and a launch in 2030, as the fifth pill of its kind to market.
2. **The price of the product collapsed before the trial started.** Lilly's pill was approved on 2026-04-01 and immediately sold at \$25/month to insured patients and \$149–349/month cash-pay depending on dose. Novo's pill launched in January 2026 at the cheapest cash price the category has ever seen. The revenue per patient that everyone modelled two years ago no longer exists.
3. **The company will need more money, and it will raise it into any rally we participate in.** It has \$1.34 billion, which it says lasts through end-2028 — *excluding* the cost of preparing to sell a drug. It also has an untouched \$400 million at-the-market share-sale program sitting over the stock.

Netting those out: this is a good drug in a bad-priced category, owned by a company that runs out of money the same quarter its decisive data arrives. At \$47.34 you are paid about fairly for that. You are not paid *well*.

What the market believes

The stock is down 50% from its 52-week high of \$94.90 and up 156% from its low of \$18.49. That shape tells you the market already lived through the repricing — this is a halved former momentum name, not something nobody has looked at.

Published analyst targets average **\$110.91**, with a wide spread: HCW cut from \$100 to \$70, Citi sits at \$97. The number doing the work in those models is peak annual sales of \$3–4 billion. Ours is \$1.3–2.4 billion, because we use the *post-April-2026* prices actually being charged rather than the prices that existed when those models were built. We think a meaningful part of that \$110.91 average is stale marks that predate the price collapse, but we could not establish that for every contributor and we say so below.

Separately, the market appears to assign **zero value** to the company's second drug, an oral amylin agonist in Phase 1 — and so did both sides of our internal debate until one of them caught it.

Why we might have edge

The takeover floor is much higher than a first read suggests. Pfizer paid **\$10 billion** for Metsera, a company whose lead drug was an *injectable* about to begin Phase 3, plus a Phase 1 amylin and a Phase 1 oral. Roche paid \$1.65 billion up front (\$5.3 billion total) for a Phase 2 amylin. Structure, at roughly **\$2.1 billion** net of its cash, owns a Phase-3-started, best-in-class *oral* plus the most advanced oral amylin in the industry. Novo lost the Metsera auction and still has no oral small molecule. Every comparable transaction sits above where this trades.

The liver question is closed and we do not think that is fully priced. The entire class carries the memory of Pfizer's danuglipron, which died on liver toxicity. Structure has now dosed 625+ people with no drug-induced liver injury and discontinuation rates of 3.7% (Phase 2b, higher doses) and 2% (extension). The market has watched one oral GLP-1 after another fail on tolerability; a clean one is worth more than a discount rate implies.

Two dated, unscheduled catalysts sit in front of us, not in 2028. The company guided 72-week extension data and the Phase 1 amylin results to the second half of 2026. Either can print on any morning. Neither is on a calendar the market can position around.

And the honest counterweight: none of these are secret. The registry date is public. The comparable deals are public. This is a name with real coverage and real dispersion — we are not the only ones who have done this arithmetic.

Why it might be priced in

The bear case here is **consensus**, not discovery. HCW cut its target to \$70 on precisely the pricing argument we make. The stock has already halved. The trial date is one registry lookup away from anyone. When we ask “what do we know that the price does not,” the honest answer for most of our findings is: nothing.

Our valuation says the same thing arithmetically. Taking the scenario set below at our own probabilities, expected value is **\$49.6 per share against a \$47.34 tape** — about 5% of upside, which is inside the error bars of every assumption feeding it. A fairly-priced asset with a two-year fuse and a guaranteed share issuance in the middle is not an edge; it is a coin flip you pay a spread to enter.

The one place we may genuinely differ from the tape is the takeover floor — but that is a tail we cannot handicap from outside, and we should not pretend a 15% probability we invented is knowledge.

What could go wrong

Failure narrative 1 — the tolerability number regresses and takes the thesis with it. The 3.7% discontinuation rate that anchors the “clean drug” story comes from **one patient out of 27**. There is no confidence interval around that. The earlier, faster titration schedule produced 10.4%. In a 3,600-patient Phase 3 the true number will land somewhere in between, and if it lands near 10% the “best-in-class *and* best-tolerated” claim narrows to “roughly the same as everyone else’s.” *Tripwire: exists but slow* — the 72-week extension data, then Phase 3 safety updates. This is monitorable, and it is the failure we would see coming.

Failure narrative 2 — the price war goes deeper than the maintenance dose. Our valuation now rests on patients paying \$2,400/year, which is the *observed maintenance* cash price. That price is two years old and set by two companies competing for share. If Lilly and Novo push net realized price toward the \$900/year end — and both have every incentive to lock the category before a fifth entrant arrives — the base case drops from \$79.90 to \$21.50 per share and no clinical result rescues it. *Tripwire: exists* — Lilly and Novo disclose obesity net price movement quarterly, and pharmacy-benefit formulary decisions are public.

Failure narrative 3 — NO MONITORABLE TRIPWIRE — the class resets from outside the company. The decisive datum is 24 months away, which means 24 months of pure narrative in between. In that window the thing most likely to destroy this position is an event with **no issuer-side observable at all**: a rare liver or oncology signal emerging in a *competitor’s* marketed oral, generalized by regulators into a class-wide monitoring requirement or REMS that lands on a drug we own but did not cause; or, symmetrically, Lilly or Novo announcing an oral GLP-1/amylin *combination* that makes a single-mechanism oral pill commercially obsolete before it ever launches. Neither shows up in Structure’s filings, its trial registry, its Form 4 cadence, or any price gate we can arm. The tape moves first and the explanation arrives second.

Unknown-unknown class, named: EXOGENOUS CLASS RESET — a third party’s data or regulatory action revalues our asset with zero observable on our issuer. We have no sensor for this class and we should stop pretending a filing-watch substitutes for one. The only defence is position size and the fact that we own nothing today.

The reflexive and structural leg.

Our own size is not the problem. At a 0.4–0.8% starter this is a few thousand shares against a name that trades a nine-figure notional daily — well under 1% of a day’s volume. We flag that our volume figure is estimated, not pulled (item 8 below), but no plausible value makes our footprint matter.

The company’s size is the problem. The \$400 million share-sale program is untouched. At \$47 that is about 8.5 million shares — roughly 12% of the shares outstanding. At \$71 it is 5.6 million, about 8%. **We would be buying a stock whose issuer has a standing, pre-registered incentive to sell into exactly the rally our thesis predicts.** Our own doctrine says never bid into flow you are forecasting; here the forecast flow is the issuer’s, it is already documented in a filed prospectus supplement, and it is triggered by our being right. The December-2025 buyers who paid \$65.00 are down 27% and are the natural supply on any move back up.

Doctrine interactions, stated plainly. Three of our own rules point the same way and we are following them rather than arguing around them: this is human-gated development-stage biotech, so nothing stages regardless of how the debate resolves; 2026 is a tax-harvest year and this position’s loss branch would not land in it; and the holding period required to reach the decisive data (September 2028) is two years, which makes any near-term exit short-term-taxable on a thesis that has not yet resolved.

If this position loses money, the most likely reason will be that we bought a clinically excellent drug into a category whose price had already collapsed, and were then diluted at the bottom by the company’s own share sale while waiting two years for data that arrived after the economics stopped mattering.

What we checked ourselves

Claim	How checked	Found	Confirmed?
Pivotal trial reads out Sept-2028	We queried the trial registry API directly rather than trusting either bench	ACCOMPLISH-1, recruiting, started 2026-07-13, primary completion September 2028 , completion Nov-2028, n=3,600, week-76 endpoint, sponsor Gasherbrum Bio (Structure subsidiary)	YES — independently verified
Trial is attributed to the right company	Sponsor field on the registry record	“Gasherbrum Bio, Inc., a wholly owned subsidiary of Structure Therapeutics” — not a mis-binding	YES
Company value net of cash	Share count from the 10-Q; 3 ordinary = 1 ADS	213,929,572 ordinary + 4,615,386 warrant shares = 72.85M ADS; \$3.449B market value – \$1.343B cash = \$2.11B	YES — and our first bench got this wrong , publishing \$2.17B. Arithmetic slip, caught internally
Lilly’s pill priced the category at \$1,800/yr	Checked the actual dose-by-dose price schedule	Our first bench priced the category off the starting dose. \$149/month is the	NO — bench error, corrected

Claim	How checked	Found	Confirmed?
		titration dose; maintenance runs to \$349/month = \$4,188/yr . This single error inverted the valuation conclusion	
Company hid the true efficacy number behind a “placebo-adjusted” figure	Read the source press release	Both the placebo-adjusted 16.3% <i>and</i> the absolute 15.3% appear in the same company release. The confusion came from secondary media re-printing it wrong	NO — bench error. Our disclosure-consistency detector should not have fired; downgraded to a review note
Efficacy gap narrows once you match trial durations	Compared timepoints	Our first bench ran the adjustment backwards. GPCR is 15.3% at 44 weeks and still improving (16.2% at 56); Lilly’s 12.4% is at 72 weeks. Matching durations <i>widens</i> the gap to ~4–5 points	NO — bench error, corrected
Stock is 20–40% overvalued	Re-did the discounted math	The bear valuation applied a 70% success probability to the value <i>and</i> separately subtracted an unsourced \$0.8B financing need — double-counting. Corrected, the bear’s own base case gives \$50.10 vs a \$47.34 tape: fair, not rich	NO — bench error, corrected
No credible acquirer	Checked comparable transactions	Pfizer paid \$10B for a <i>less</i> advanced portfolio (Metsera); Roche paid \$5.3B total for a Phase 2 asset. Comparable deals sit above this tape, not below	NO — bench error, corrected
Liver toxicity risk (the vector that killed Pfizer’s version)	Company clinical data	No drug-induced liver injury, no persistent enzyme elevation, no QTc signal across 625+ exposed	PARTIAL — our second bench found the entire axis our first bench never checked , but it rests on a journal publication we could not independently open this session. Treated as strong-but-unverified (item 7)
Insider selling is a signal	Form 4 cadence	Monthly filings (6/18, 6/29, 7/15, 8/5, 8/19, 9/2) — the signature of an automated plan, not conviction selling. Size is trivial (~\$210k)	Downgraded to noise — but not to zero. Our second bench dismissed it entirely; we keep the 9/1 sale at \$47.20 on the record as data

Claim	How checked	Found	Confirmed?
“Untouched share-sale program = issuance discipline”	Re-read	Our second bench credited the company for restraint. The simpler reading is that the stock is 27% below the last raise price, so selling now is unattractive. Both readings are consistent with the facts; we do not claim to know which	NOT ESTABLISHED — we decline to credit it
Company runs out of money when the data lands	10-Q + guidance	\$1.343B, guided “through end of 2028,” explicitly excluding pre-launch spending. First-half cash burn (\$104.9M) is <i>lower</i> than one quarter’s reported loss (\$106.2M) — the trial bills have not come due yet	YES — and this is the finding that survived both benches intact
\$400M share-sale program is live	Prospectus supplement	Upsized to \$400M gross on 2026-05-07; unused through June 30	YES (424B5)
Competitors are fully framed by the company	Cross-checked pipeline disclosures	AstraZeneca’s Phase 3 oral (11.8% at 36 weeks) and Roche’s candidate are absent from the company’s competitive framing	YES — flagged as omission
Next earnings date is 2026-11-05	Checked source of the date	Vendor-derived; no company release names it . ~44 trading sessions out	NO — unconfirmed (item 6)
Options are cheap or rich	Attempted a live chain pull	Permission denied for the third time — first bench, second bench, and this deck all blocked. No chain quotes exist in this analysis	NO — open coverage gap (item 1). We do the math below off our own table and mark the comparison PLAUSIBLE, not confirmed
Second drug (oral amylin) is worth something	Reviewed both benches’ models	Both benches valued it at exactly zero . Given what Roche and Pfizer paid for early amylin assets, zero is not a neutral assumption — it is a wrong one	NO — shared bench omission , corrected into the scenario table

PROPOSED ENTRY BANDS

The courts say no entry today. Development-stage biotech is human-gated at this desk — nothing stages automatically regardless of how the argument resolves — and independently, the arithmetic does not ask us to buy. These are therefore **pre-registered reopen levels, not resting orders. Nothing is staged.**

Reopen bands (stock only):

Level	Action	Size	Why this price
≤ \$40.00	Reopen for a starter — requires a human click	0.4% of book	EV \$49.60 vs \$40.00 = +24%. This is the level at which the scenario table pays for the 2027 financing rather than hoping it away
≤ \$32.00	Scale the starter	to 0.8% of book	Company value net of cash falls to ~\$1.0B. Even the low-price scenario (\$1,800/yr, 0.75M patients, post-dilution \$51/share) pays from here
≥ \$71.00	Do not chase; treat as the issuer's sell zone	—	+50% from here is where the untouched \$400M program becomes attractive to the company. We do not bid into flow we are forecasting

Both bands assume the 2027–28 share issuance happens and are computed after a 20% dilution haircut. Neither band is a commitment; both require the decisive-datum question below to still be open when they trigger.

Conditions that would move the bands *up* (i.e. let us pay more): - A signed partnership or license with **≥\$500M disclosed up-front** — this closes the funding gap and validates the takeover floor in one event. - 72-week extension data showing **≥18% absolute** weight loss with discontinuations **under 8%** and clean liver enzymes at 72 weeks. - Evidence that net realized price for oral obesity drugs is stabilizing at **≥\$250/month** — this reopens the top scenario column.

Catalyst dates: - **On or before 2026-09-30** — 72-week extension data, guided to Q3 2026, unscheduled, can print any morning. - **Second half of 2026 (through 2026-12-31)** — Phase 1 results for the oral amylin drug, unscheduled. - **~2026-11-05** — Q3 report. Vendor-derived date, **not confirmed by the company**. The number that matters in it is the third-quarter cash burn, which tells us whether the trial bills have started landing. - **September 2028** — pivotal trial primary completion. Registry-confirmed. This is the date the thesis is actually about.

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 32.0 (deck-provisional: Scale band to 0.8%: enterprise value falls to ~\$1.0B, where even the \$1,800/yr low-price scenario (\$51)
- filing watch: 8-K,6-K until 2027-03-02 (deck-provisional: 72-week ACCESS-OLE data and ACCG-2671 Phase 1 SAD topline are both guided to open windows and can prin)
- dated pack GPCR|2026-09-30: GPCR: End of the company-guided Q3-2026 window for ACCESS-OLE 72-week data (absolute weight loss, discontinuation rate, 72-week liver enzyme

- dated pack GPCR|2026-11-05: GPCR: Q3-2026 report; the load-bearing line is Q3 cash burn versus net loss - tells us whether Phase 3 accruals have started converting to c
- dated pack GPCR|2026-12-31: GPCR: End of the guided 2H-2026 window for ACCG-2671 oral amylin Phase 1 SAD data - the asset both benches valued at zero. (date confirmed=F
- dated pack GPCR|2028-09-30: GPCR: ACCOMPLISH-1 primary completion (week-76 endpoint, n=3,600) - the datum the entire thesis is about; registry-verified this session. (d

What remains unverified

Every item below is genuinely open. Items already drained or estimated are dispositioned in the table above, not here.

1. **The options chain — real strikes, real quotes, real implied volatility.** Blocked on broker permission, three attempts. Vendor-sourced implied volatility of 73–81% is plausible but unconfirmed, and we have been burned before by a vendor's headline volatility diverging from the actual chain by 30 points. *Estimate used: 73–81%, vendor feed, confidence 0.6.*
2. **72-week extension data: absolute weight loss, discontinuation rate, liver enzymes at 72 weeks.** Company-held, unscheduled. *Estimate: 17–19% absolute weight loss, extrapolating the 15.3% @44wk → 16.2% @56wk trajectory with a decelerating slope. Probability it lands ≥18% with discontinuations under 8%: 0.40.*
3. **Phase 1 results for the oral amylin drug (ACCG-2671).** Company-held, unscheduled. *Estimate: 0.60 probability of a clean safety profile with meaningful weight loss at top dose; risked value \$0–800M, roughly \$0–11/share, base case \$5.50/share. Both benches carried this at zero.*
4. **Actual cash burn in 2027–28.** Reported losses currently exceed cash out the door, which means trial costs are accrued but unpaid — the burn re-rates upward and we do not know by how much. Partially drainable at the Q3 report. *Estimate: \$420–520M/year in 2027–28 against a first-half-2026 run-rate of ~\$210M, based on a 3,600-patient Phase 3 plus extension plus the amylin program; implied additional capital need \$600–900M.*
5. **Whether a partnership or licensing deal is under negotiation.** Not observable from outside. This is simultaneously the biggest upside tail and the thing that would close the funding gap. *Estimate: 0.30 probability of a ≥\$500M up-front deal announced by end-2027, from the base rate for Phase-3-stage best-in-class obesity assets partnering within 24 months (~25–35%).*
6. **The 2026-11-05 earnings date.** Vendor-derived; no company release names it. *Estimate: the report lands 2026-11-03 to 2026-11-12, probability 0.85.*
7. **The liver-safety publication itself.** Our second bench's decisive bull finding cites a journal paper we could not open in this session. The company's own summary is consistent with it, but we have not read the primary. *Estimate: 0.90 probability the claim holds as stated; the residual is about the precise enzyme-excursion tabulation, not the headline.*
8. **Real daily volume, free float and short interest.** Vendor figures only (short interest ~8.6–9.4%). Our position-size conclusion does not depend on these, but our "our size doesn't move it" claim technically does. *Estimate: \$100–190M average daily traded value; our largest contemplated order is under 0.5% of a day, probability 0.9.*

9. **Composition of the \$110.91 average analyst target.** We assert that much of it predates the April-2026 price collapse; two of the visible marks (\$70 and \$97) clearly do not. *Estimate: 0.50 probability that a majority of contributors to the average have not refreshed pricing assumptions post-April-2026.*

PRIZE TABLE + VOL READ

Multiple load-bearing unknowns are time-gated and private (items 2, 3, 5), so the table is required.

Anchor. Market price \$47.34. Our scenario expected value comes out at \$49.60 — a 4.8% deviation from the anchor, inside the $\pm 5\%$ band that requires no separate mechanism. Where we *do* deviate from our own benches, we name it: both used a 70% probability of clinical success; we use an effective ~62–65%, because of the finding neither bench disputed — the 3.7% discontinuation rate is one patient out of twenty-seven and will regress in a 3,600-patient trial.

All prices per share, after a 20% dilution haircut on the drug's value (the 2027–28 raise is assumed, not hoped away). Cash is \$17.57/share today.

#	Scenario	p	Price/share
A	Pivotal fails, or tolerability forces a label that kills commercial value	0.32	\$6.00
B	Approved; price war deepens to ~\$900/yr, 0.30M patients	0.10	\$21.50
C	Approved; ~\$1,800/yr, 0.75M patients	0.20	\$51.20
D	Approved; observed maintenance blend ~\$2,400/yr, 1.0M patients	0.17	\$79.90
E	Approved; ~\$3,000/yr, 1.5M patients	0.06	\$137.60
F	Acquired before readout, at a discount to the Metsera/Roche comps	0.15	\$90.00

Expected value = \$49.65 (vs \$47.34 tape, **+4.8%**). **Dispersion of the table: $\sigma = \$39.40 = 83\%$ of spot**, over the ~2.3 years to the September-2028 readout.

Note the shape: **this is bimodal, and severely so**. 32% of the probability sits at \$6 and 38% sits above \$79. There is very little mass near the current price. A fairly-priced bimodal asset is not the same thing as a safe one — the expected value is 5% above the tape and the modal outcome is a 87% loss.

Expression math — is there a better instrument than the stock?

Converting the table's terminal dispersion to an annualized-equivalent volatility: $0.83 \div \sqrt{2.3} = \sim 55\%$. Solving the same thing the other way — the volatility at which a standard option model reproduces our own table's call values — gives $\sim 53\%$. The two agree.

Pricing two candidate strikes directly off the table, assuming an expiry that spans the September-2028 readout:

Strike	Our value from the table: $\sum p_i \times \max(S_i - K, 0)$	Discounted @4%, 2.3yr	Model value at vendor IV 75%
\$50	\$16.58	\$15.12	\$20.89
\$70	\$8.74	\$7.97	~\$14.60

The options are approximately 38% more expensive than our own table says they are worth, and the gap is ~ 20 volatility points (our 53–55% versus the vendor's 73–81%). Realized volatility, proxied from the actual 52-week path (\$18.49 $\rightarrow$ \$94.90 $\rightarrow$ \$47.34), runs ~ 70 – 90% .

So: **volatility risk premium ≈ 0 to -10 points** — implied volatility is at or *below* realized. That means:

- **Buying calls is rejected.** They cost $\sim 38\%$ more than our scenario table values them at. Cheap optionality on a genuinely uncertain fork would be a reason to express the gated adds in calls; this is the opposite case.
- **Selling premium is also rejected**, and for the correct reason — not squeamishness, but arithmetic. There is no volatility risk premium to harvest (implied is not above realized), and independently this fails our premium-selling gates outright: a cash-secured put requires a downside floor at or above the strike, and scenario A's \$6 says no such floor exists. A binary clinical fork is precisely the situation those gates were written to exclude.
- **The instrument recommendation is therefore: NONE — no stock, no options.** That is the output of the arithmetic above, not a preference.

Honest caveat on all of the above: the volatility we are comparing against is **vendor-sourced and unconfirmed** — chain access was denied to both benches and to this deck. Our own doctrine records a case where a name's chain-level volatility read 71 against an aggregate feed's 101. If the real chain prices near 55%, the \$50 strike becomes roughly fair and the calls-versus-stock question genuinely reopens. **We are not making the call; we are recording that we could not make it, and naming exactly what would settle it.**

Implied volatility percentile: NOT AVAILABLE — same blocked lookup.

Tax overlay

2026 is a harvest year for this book. The loss branch of this position — scenario A, at 32% probability, the single most likely individual outcome — needs to land in the tax year we intend to use it.

- **A stock position entered today** does not resolve until at least the 72-week extension data and realistically not until September 2028. Its loss branch lands in 2027 or 2028, not 2026. It is also short-term-taxable for the first twelve months, on a thesis that cannot resolve inside twelve months. **Wrapper mismatch.**
- **Any option long enough to span the September-2028 readout** expires in 2029. Its loss defers three tax years. Under harvest-year rules that is excluded unless the edge numerically dominates — and the edge here is *negative* (we would be paying 75% volatility for a 55% distribution). **Excluded on both counts.**
- **A short-dated option positioned for the 2H-2026 catalysts** would land its loss in 2026 correctly, but it is not the thesis — it is a bet on the timing of an unscheduled press release, and it is being sold at 73–81% volatility against 70–90% realized. No edge, wrong question.

The tax overlay and the volatility arithmetic point at the same door. **No position.**

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$5.50/ADS", "basis": "Roche/Zealand $1.65B upfront for a Phase-2 amylin and
Pfizer/Metsera $10B including Phase-1 amylin, discounted for Phase-1 stage
and single-ascending-dose-only data", "p": 0.6}}, {"item": "Actual 2027-28
cash burn - reported losses currently exceed cash out the door, so Phase 3
accruals are unpaid and the burn re-rates.", "instrument": "Q3-2026 10-Q
(~2026-11-05) operating cash flow line vs net loss; XBRL ocf pull each
quarter", "drainable_now": false, "estimate": {"range": "$420-520M/yr in
2027-28; implied incremental capital need $600-900M", "basis": "H1-2026 run-
rate ~$210M/yr scaled for a 3,600-patient Phase 3 plus OLE plus the amylin
program, against guidance of cash through end-2028 EXCLUDING pre-
commercialization", "p": 0.65}}, {"item": "Whether an ex-US or global
partnership / licensing deal is under negotiation - closes the funding gap
and the takeover floor simultaneously.", "instrument": "8-K/PR watch; SC 13D
watch; conference-presentation and JPM-window monitoring", "drainable_now":
false, "estimate": {"range": "0.30 probability of a >=$500M disclosed upfront
announced by end-2027", "basis": "base rate for Phase-3-stage best-in-class
obesity assets partnering within 24 months of pivotal start (~25-35%)", "p":

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0.3}}, {"item": "The 2026-11-05 earnings date is vendor-derived and NOT confirmed by any company release.", "instrument": "Company IR press release / 8-K announcing the earnings date", "drainable_now": false, "estimate": {"range": "2026-11-03 to 2026-11-12", "basis": "issuer's own prior-quarter reporting cadence (Q2 filed 2026-08-06)", "p": 0.85}}, {"item": "The primary hepatotoxicity publication underlying the decisive bull finding – could not be opened this session.", "instrument": "Direct fetch of the Nature Medicine ACCESS-2b paper; cross-check against the company's own clinical data disclosures", "drainable_now": true, "estimate": {"range": "0.90 probability the headline claim (no DILI, no persistent enzyme elevation, no QTc signal across >625 exposed) holds as stated", "basis": "consistent with the issuer's own primary disclosures in the ACCESS-II topline PR; residual uncertainty is the enzyme-excursion tabulation, not the headline", "p": 0.9}}, {"item": "Real average daily traded value, free float and short interest – vendor figures only.", "instrument": "IBKR get_price_snapshot (volume, avg_90d_usd_volume) and get_price_history; short interest from the exchange semi-monthly file", "drainable_now": true, "estimate": {"range": "$100-190M average daily traded value; short interest 8.6-9.4% of float", "basis": "vendor short-interest feed; ADV inferred from a $3.45B cap halved-momentum name with active options interest", "p": 0.7}}, {"item": "Composition of the $110.91 average analyst price target – how much of it predates the April-2026 oral GLP-1 price collapse.", "instrument": "Broker estimate feed with per-analyst mark dates; or count post-2026-04-01 published notes directly", "drainable_now": false, "estimate": {"range": "0.50 probability that a majority of contributors have not refreshed pricing assumptions post-April-2026", "basis": "two visible marks (HCW $70, Citi $97) are confirmed post-collapse and both sit far below the $110.91 mean, implying stale high marks in the tail", "p": 0.5}}}]}
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Three things worth saying plainly about this deck. First, the strongest work in it came from our second bench catching that our first bench had priced an entire drug category off the starting dose rather than the maintenance dose — an error that, uncorrected, would have produced a confident and wrong rejection. Second, both benches independently valued the company's second drug at zero, which is not a conservative assumption but a wrong one, and it took a third pass to notice. Third, the reason we are not buying is not that the debate resolved bearish — it resolved roughly *fair*, which is a different and more boring answer: we would be paying full price for a bimodal outcome, two years early, into a share sale the company has already filed for.

ADJUDICATION STAMP — 2026-09-04 (FABLE)

NO ENTRY — WATCH, pre-registered. HUMAN-GATED CLASS: nothing stages regardless of any future branch; band-touch = notify + re-court, never an order. The marketed best-in-class premise fails two structural tests: the 16.3% headline is an on-treatment estimand from the N=85 safety-primary study with a weight-gaining placebo (per-arm N never disclosed), while the powered study's -11.3% sits level with orforglipron and oral semaglutide per independent NMA — and the 2.0-3.4% tolerability story is a 20-week interim against competitors' completed 72-week label rates. Against that: the hepatotox class-vector is closed, and aleniglipron is the sole independent Phase-3 oral small-molecule GLP-1 in existence with ten majors holding none (Metsera auction precedent; Lang + Berrios hires read as both-paths-open). Bench conflict resolved at primary: oral semaglutide IS approved for obesity (NDA 218316, 12/22/25) — two approved orals, not one. Reopen frame: $\leq \$40$ AND OLE-72wk mature discontinuation $< 5\%$, or a $\geq \$500\text{M}$ -upfront partnership. Q3 pack armed with framing tripwires (the decisive line is the discontinuation number,

not the open-label weight number; SAD-without-MAD is the tell). Options chain verified untransactable (OI in tens, 20–55% spreads) — no expression edge either way. Prize table: bear 0.35 / base 0.45 / bull 0.20, $EV \approx +6\%$, right tail entirely M&A. Full ruling: [desk/data/court_artifacts/GPCR_ADJUDICATION_202609040415.md](#).